ADVANCING WHOLE PERSON DIABETES CARE: FEASIBILITY OF EMBEDDING SOCIAL NEEDS SCREENING INTO DIABETES SELF-MANAGEMENT EDUCATION DELIVERY

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01 INTRODUCTION

- Diabetes self-management education (DSME) programs improve health behaviors
- Health-related social needs (HRSN) (e.g., food insecurity, housing instability, transportation) hinder DSME engagement and health outcomes
- DSME programs reach populations at-risk for health disparities, but most do not systematically assess HRSN, limiting their effectiveness
- These challenges are global in scope and disproportionately affect hard to reach populations

02 OBJECTIVE

To assess the feasibility of integrating HRSN screening and referral pathways into DSME delivered in community-based settings.

Arizona State University IRB reviewed and granted exempt status. No funding supported this work.

03 METHODOLOGY

Design: Qualitative multiple case study

Data Collection: Semi-structured individual interviews

Participants: 15 DSME facilitators and organizational leaders at three urban and rural community-based organizations in the USA

Analysis: Thematic coding (inductive followed by deductive); Guided by the Consolidated Framework for Implementation Research

Focus: Feasibility, barriers, facilitators, and contextual determinants

04 RESULTS

Challenges in Current DSME Structure

- Rigid DSME structure limits flexibility to address real-time HRSN
- Absent screening and referral processes leaves facilitators unsure how to respond to clients
- Added responsibilities without resources strain capacity
- Unmet HRSN drive attrition and may exclude more vulnerable clients

Opportunities for HRSN Screening

- DSME intake and enrollment = practical points for brief HRSN screenings and referral planning
- Community partnerships = shared responsibilities and strengthen sustainability
- Build upon community trust = engagement and retention

> "THEY STOP COMING, AND WE DON'T KNOW WHY. YOU START TO REALIZE IT'S MORE THAN JUST INTEREST—IT'S SURVIVAL."
> - DSME FACILITATOR

05 CONCLUSION

- Integrating HRSN screening into DSME workflows is feasible and valuable while maintaining program fidelity
- Training navigators/DSME facilitators supports early identification of HRSN and referrals
- Cross-sector partnerships and reimbursement mechanisms are key to sustain integration
- CBOs are an ideal setting to address HRSN alongside providing DSME support

Full Publication

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